



Granting and Management of Leave for Mental Health Inpatients

Policy code	SMHS:11
Original endorsement date	01 September 2003
Date of current endorsement	02 February 2024
Functional subgroup	Mental Health
Summary	This policy sets out the provisions for the granting leave to mental health inpatients within the South Metropolitan Health Service (SMHS).
Policy owner	Executive Director Clinical Service Planning and Population Health, Kate Gatti.
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Review date	02 February 2027
NSQHS Standards	Standard 1: Clinical Governance Standard 2: Partnering with Consumers Standard 5: Comprehensive Care
Chief Psychiatrists Standards for Clinical Care Status	Care Planning Consumer and carer involvement in individual care Active

1. Background

1.1. About this document

Leave for admitted mental health inpatients is encouraged to support person-centred recovery, sustain community-based living and ensure that treatment is provided in the least restrictive manner possible. It may also be a key component of discharge planning.

1.2. Key Definitions

Term	Definition
Absent person	A mental health patient whose whereabouts should be known but cannot be ascertained after diligent efforts are made to locate and/or contact them have been made by staff however their life or health is not believed to be in danger, and they are not believed to be at risk of harming others.
Absent without leave (AWOL)	Where an involuntary patient is absent without approved leave from a hospital or other place (MHA 2014 s.97).
Escorted Leave	A leave of absence, where an individual is appointed to consistently accompany the inpatient throughout their leave period.
Involuntary patient	A person who is under an involuntary inpatient treatment order or an involuntary community treatment order (MHA 2014 s21).
Missing person	Any voluntary psychiatric patient at high risk of harm who is missing from a mental health service, general hospital or emergency department, without the agreement of or authorisation by staff.
Personal Support Person (PSP)	Can be any of the following people who are supporting someone who is experiencing mental illness - close family member, carer, nominated person, the parent or guardian of a child, any guardian or enduring guardian of an adult (MHA 2014 s.7).
Voluntary patient	A person who is receiving treatment by a Mental Health service voluntarily.

2. Policy Statement

Decisions to support, or decline, leave requests are based on the assessment and management of clinical risk and should be made through active collaboration with the treating team, the patient, carers and families. Decisions regarding a patient's leave and the rationale behind decision making is to be documented in full in the patient's Digital Medical Record (DMR). This document outlines the requirements to be met when granting leave to mental health inpatients. Roles and Responsibilities

2.1. Executive

Each SMHS site Service Director/ Co-Director will ensure that managers and supervisors are aware of the requirement to implement safe practices relating to granting and management of leave for mental health inpatients.

2.2. Managers/supervisors

Managers and supervisors should:

- Ensure the development of relevant protocols and procedures.
- Ensure staff are trained in relevant safe Mental Health practices.
- Where available and appropriate, ensure appropriate training and education is provided to staff in relation to granting and management of leave for mental health inpatients.

3.3 Staff

Staff should:

- Ensure patients and carers are provided the opportunity to discuss leave arrangements prior to leave
- Document any leave taken as well as the patient's overall leave experience in the patient's Digital Medical Record (DMR)
- Comply with mandatory training requirements and proactively seek further training as required
- Comply with safe Mental Health practices
- Ask for assistance if required and involve relevant medical staff in the decision-making process regarding granting and management of leave for mental health inpatients.

3. Risk assessment

This assessment, together with any risk mitigation strategies identified must be documented in the patient's DMR at the earliest available opportunity, preferably prior to any leave commencing.

Consideration should be given to the patient's:

- risk to self.
- risk to others.
- risk from others.
- physical health status.
- risk of absent without leave.
- current level of observation within an inpatient setting
- Mental Health Act status

It is recommended that newly admitted involuntary patients are not initially granted leave, even for

brief periods, until the treating team has developed enough familiarity with them to make a valid assessment of their mental state and risk.

Risks related to leave will be weighed against the positive benefits of empowering the patient to take more responsibility and control of their life and recognition of the cultural, diverse, and individual needs of the patient, carer and family. Risk mitigation strategies will be documented and shared with the patient, carer and family prior to the commencement of leave.

When assessing the granting of leave and determining an appropriate escort, the multidisciplinary team is to consider the role of the patient and escort. Information will be gathered from stakeholders involved including the patient and family.

Regardless of the legal status of the patient under the Mental Health Act 2014, the allocated nurse and / or treating medical officer will review the patient's current risk and mental state examination before each episode of leave in collaboration with the patient, carer/s and / or escort where appropriate. Refer to the [Mental Health Act 2014](#), s. 105, 108-110.

4. Granting leave

Refer to the [Mental Health Act 2014](#) s. 105, 108-110.

Refer to the following policies/procedures:

- [SMHS Missing or Suspected Missing Mental Health Patients Policy](#)
- [Prevention and Management of Workplace Aggression and Violence Policy](#)
- [MP 0012/16 Missing Person Policy](#)
- [RKPG Patient Leave c:017](#)
- [FSFHG Mental Health patient leave](#)

5. Voluntary patients

Voluntary patients are free to leave the hospital premises. At the earliest opportunity in the inpatient episode, staff are to communicate to voluntary patients the local processes and general responsibilities held by both patients and staff for supporting safety during periods of leave. Patients and carers are encouraged to inform staff when they are taking periods of leave. Staff are to ensure patients and carers are provided the opportunity to discuss leave arrangements prior to leave, on any specific occasion of leave.

Leave for voluntary patients is to be granted as per local site policy /procedures. All absences from the ward should be documented in the patient's DMR to facilitate staff awareness of any absence and for information in the event of an internal emergency.

Voluntary Patients who unexpectedly depart the Mental Health Unit without notifying staff will have a risk review performed (including attempts to contact the patient to verify safety).

Where the voluntary patient is assessed as elevated risk, action is to be taken in line with the [SMHS Missing or Suspected Missing Mental Health Patients Policy](#).

6. Involuntary patients

Involuntary patients may only be granted leave of absence by a consultant psychiatrist. The psychiatrist's decision to grant leave should consider acute and chronic risks and include the views of the patient, carer/s, family and Multidisciplinary Team.

The grant of leave may be for escorted or unescorted leave.

All episodes of leave (day leave or overnight) for involuntary patients should be documented in accordance with the Mental Health Act 2014 and documented on a Form 7A Grant of leave to an involuntary patient, with the PSP notified.

The grant of day leave for involuntary patients will be documented in the patient's DMR, in the manner prescribed by local policy or procedure, and must include the period of leave granted, the frequency of the leave, and any restrictions that may be necessary. A plan should be documented of what is to be done if the patient fails to return.

All episodes of leave must be documented contemporaneously (i.e., at the time) for the safety of the individual patient and for information in the event of an internal emergency.

When considering whether to grant a leave of absence to an involuntary patient, a psychiatrist should consider whether voluntary status or a Community Treatment Order (CTO) would be more appropriate.

7. Escorted leave

If the psychiatrist authorises escorted leave, then the person(s) considered suitable to provide the escort must be specified by name or by role in the grant of leave.

The appointment of an escort should be consistent with the wishes of the patient unless the:

- nominated escort does not consent to escorting the patient; or
- psychiatrist is concerned that the person nominated as escort is likely, based on current risk assessment, to be placed in a situation they are unable to manage during

leave.

The patient's next of kin / carer / support person or another identified responsible person can escort the patient at the treating psychiatrist's discretion, subject to them agreeing to the conditions of the leave.

The escort is to be provided information with regards to the expectations of their role. This is to include awareness of:

- The duration of leave and any conditions placed on it.
- The need to stay with the patient for the duration of the leave.
- The need to be able to contact the hospital, and to be contacted, by mobile phone during the leave.
- Feeding back to nursing staff on return of the patient to the ward.

The nominated individual has the right to decline being an escort. If the proposed escort person does not agree to the above conditions, the grant of leave may be withdrawn until alternative arrangements are made.

7.1. Escorted leave by clinician

Infrequently, it may be necessary or desirable for a clinician to conduct the escort. In this instance:

- Handover must take place before and after all leave escorted by a clinician.
- Clinicians should carry a phone with them at all times with the Safe Haven app

Escorted leave by a clinician will be provided in the manner prescribed by local policy or procedure and is subject to:

- The patient's needs for supervision.
- The capacity of the service to provide an escort, given the existing or anticipated clinical acuity.

8. Staff consultation

The patient's allocated nurse or the Shift Coordinator must be consulted before any patient is escorted from the ward by any staff member of any profession in compliance with local policy.

Mental health clinicians will exercise their discretion to decline leave that has been granted, pending review by a doctor, if it is assessed that there has been a change in the level of assessed risk or an adverse change in the patient's mental state or physical condition.

A refusal of leave can precipitate distress, threatened or actual self-injury, aggression and absconding. If leave is not considered appropriate staff will:

- Communicate this decision to the patient using Safe wards interventions at an appropriate time and in a suitable setting.
- Develop alternative strategies to address the patient's expressed needs (e.g., if spending money is required, a family member can be asked to bring some to the ward).

This situation, and the rationale for declining approved leave, is to be documented in full in the patient's DMR and discussed with the patients and carer / family, where appropriate.

9. Review of patients on return from leave

Following the patient's return from leave, staff will routinely ask the patient how they felt the period of leave went:

- Did the patient feel the period of leave taken was successful?
- How the patient felt whilst they were on leave? Did they feel safe?
- Would the patient be confident if they were to have another period of leave?
- If appropriate, ask the patient's personal support person/family how they felt the leave went.
- Document feedback in the integrated notes.
- Be aware that alcohol and/or illicit substances may be used and take appropriate action

The Ward Clerk will re-admit the patient back into the Web-based Patient Administration System (WebPAS).

10. Documentation

The responsible mental health clinician must document any leave taken at the time of leave in the patient's DMR.

- Documentation must be contemporaneous, and include:
- the patients mental state immediately prior to leave
- the time of going on leave.
- the time of returning.
- Handover of leave arrangements to the next shift where applicable
- Any notable incidents / events which are reported to have occurred during the leave, including any deterioration in the patient's physical health status or mental condition
- Where applicable, the reason for the patient not returning by the agreed time

11. Patient missing or AWOL or fails to comply with granting of leave conditions

If the patient goes missing whilst on leave, fails to comply with any conditions attached to the grant of leave or is missing from the inpatient ward, this must be reported immediately to the Ward Coordinator / Clinical Nurse Specialist or Nurse Unit Manager and the responsible medical staff.

Regardless of the patient's status under the Mental Health Act 2014, if a mental health inpatient on leave does not return at the agreed time and cannot be contacted to establish their safety, the inpatient will be considered as missing, based on the clinician's risk assessment. In any such circumstance, interventions as prescribed in the SMHS Missing or Suspected Missing Mental Health Patient Policy will be initiated in line with local policy and procedures.

Refer to:

- [SMHS Missing or Suspected Missing Mental Health Patient Policy](#)
- [RKPG Missing or Suspected Missing Patients AC: 208](#)

12. Compliance monitoring and legislative penalties

12.1. Compliance monitoring

Each SMHS site or service director is to ensure compliance with this policy. Compliance monitoring methods may include:

- Monitoring of clinical incidents related to missing or AWOL mental health patients.
- Review of critical incidents that occur whilst a patient is on a period of granted leave.

12.2. Legislative obligations

There are no legislative obligations relating to this policy for which a statutory penalty is imposed for a breach.

13. Legislation

- [Mental Health Act 2014](#) sections Part 7, Division 5 and Division 6

14. Additional references

14.1. Department of Health policies

- [MP.0012/16 Missing Person Policy](#)
- [MP 0165/21 Use of Physical and/or Mechanical Restraint during Road-based Transportation of Mental Health Patients Policy](#)

14.2. SMHS policies

- [SMHS Missing or Suspected Missing Mental Health Patient Policy](#).
[SMHS Prevention and Management of Workplace Aggression and Violence Policy](#)

14.3.Document Control

Version	Published date	Effective from	Review date	Effective to*	Amendment (s)
GP 5	05/02/2024	02/02/2024	02/02/2027	2027	Original version

16.4. Approval

Approval by	Executive Director Clinical Service Planning and Population Health, Kate Gatti
Approval date	02 February 2024

This document can be made available in alternative formats on request.

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Compiled: South Metropolitan Health Service 2023

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